

SBAR Templates & Communication Scripts

How to Talk to Doctors, Patients, and Families Like an Experienced Nurse

By Nnamdi Okorafor, RN

The SBAR Framework

SBAR isn't just a communication tool. It's how you organize your thinking before you speak. Master this, and you'll sound like a nurse with 10 years of experience — even on day one.

TEMPLATE 1: Routine SBAR

Use For:

- Scheduled provider updates
- Non-urgent changes in condition
- Care plan discussions
- Discharge planning

Template:

SITUATION "I'm calling about [Patient Name] in room [Number]."

BACKGROUND "[Age]-year-old [gender] admitted on [date] for [diagnosis]." "Relevant history: [comorbidities]." "Current status: [stable/improving/declining]."

ASSESSMENT "Vitals: [BP, HR, RR, SpO2, Temp]." "Physical assessment: [key findings]." "Labs: [relevant values, trends]." "My assessment is [your clinical judgment]."

RECOMMENDATION "I'd like to [your request]." "Are there any orders you'd like me to implement?"

Example:

S: "I'm calling about Mr. Johnson in room 4B."

B: "72-year-old male, post-op hip replacement on day 2. History of hypertension and Type 2 diabetes."

A: "His vitals are stable — BP 138/82, HR 78, RR 16, SpO2 96% on room air. He's tolerating diet, pain is 3/10 controlled with PO meds. However, his Foley was removed this morning and he hasn't voided in

6 hours. Bladder scan shows 450mL.”

R: “I’d like to bladder scan again in 2 hours. If still retained, may need to straight cath. Should I scan and call you back, or do you want to order a straight cath now?”

TEMPLATE 2: Urgent SBAR

Use For:

- Significant change in condition
- Abnormal labs requiring intervention
- Patient request that needs provider input
- Concerns about current plan of care

The Difference:

Lead with what you need. Don’t bury the urgency.

Template:

SITUATION “I need you to see/order something for [Patient Name] in room [Number].”

BACKGROUND “[Brief background — keep it short]”

ASSESSMENT “The change is: [specific, measurable].” “My concern is: [clinical significance].”

RECOMMENDATION “I need [specific order/request].” “This is urgent because [reason].”

Example:

S: “I need you to come see Mrs. Smith in room 8. I think she’s developing sepsis.”

B: “68-year-old, post-op colectomy, day 3.”

A: “Her temp is 102.4, HR 118, BP 94/58. She’s confused and not making sense. WBC is 18,000.”

R: “I need broad-spectrum antibiotics and possibly a transfer to ICU. Can you come now?”

TEMPLATE 3: The “I Need Orders NOW” SBAR

Use For:

- Rapid response situations
- Before a code
- Critical values

- Situations where delay harms the patient

Template:

SITUATION "This is urgent. [Patient Name], room [Number] — [critical finding]."

BACKGROUND "[One sentence background]"

ASSESSMENT "[Critical vital signs/labs]." "This means [clinical significance]."

RECOMMENDATION "I need orders for [specific interventions] NOW." "I'm starting [what you're doing immediately]." "I'll call back in [timeframe] with update."

Example:

S: "This is urgent. Mr. Davis, room 12 — he's having chest pain and his EKG shows ST elevation."

B: "54-year-old, admitted for pneumonia."

A: "BP 168/94, HR 102, complaining of chest pressure. EKG shows ST elevation in inferior leads. I suspect STEMI."

R: "I need aspirin, nitroglycerin protocol, and cardiology activation. I've applied O2 and obtained IV access. Cardiology needs to be called now."

TEMPLATE 4: Family Communication Scripts

Delivering Bad News

Never say: - "I have bad news" (too blunt) - "Everything will be fine" (false reassurance) - "I don't know" (without offering to find out)

Instead say: "I need to talk to you about [patient's name]. The doctor asked me to update you on what's happening."

Then: - Use clear, simple language - Pause for questions - Offer to get the provider for detailed discussion - Provide emotional support (sit down, make eye contact)

Example:

"Mrs. Johnson, I need to update you on your husband. He's had a change in his condition. His heart rhythm became irregular about 20 minutes ago. The medical team is with him now, and they've started medications to stabilize it. The doctor will come talk to you in person as soon as they can. I know this is scary. Can I get you some water? Is there someone I can call for you?"

TEMPLATE 5: The Difficult Patient/Family

When a Family Member is Angry

Step 1: Acknowledge the emotion “I can see you’re frustrated. I would be too.”

Step 2: Identify the specific concern “Help me understand exactly what’s concerning you.”

Step 3: Explain what you’re doing “Here’s what we’re doing and why...”

Step 4: Offer choices when possible “Would you like me to get the doctor now, or would you prefer to wait until rounds?”

Step 5: Set boundaries if needed “I want to help you, but I need us to communicate respectfully.”

The “Let Me Talk to Your Supervisor” Script

“I understand you’d like to speak with my supervisor. I’m going to get them now. While I do that, can you tell me specifically what happened so I can brief them accurately?”

Never: Get defensive, argue, or refuse. **Always:** Escalate, document, and debrief.

TEMPLATE 6: Handoff SBAR (End of Shift)

The Perfect Handoff

S: “I’m giving you report on [Patient Name], room [Number].”

B: “[Age], admitted [date] for [diagnosis]. Past medical history: [relevant]. Surgery/procedures: [if any].”

A: “Current vitals: [vitals]. Assessment findings: [key points]. Active issues: [what’s being monitored/treated].”

R: “What needs to happen this shift: [tasks, meds, assessments]. Watch for: [potential problems]. Call provider if: [parameters].”

Handoff Red Flags (Always Mention)

- Fall risk patients
 - Patients with new orders
 - Abnormal vitals trending
 - Family concerns/complaints
 - Patients who are “sundowning” or confused
 - Isolation precautions
 - Wounds/drains/tubes
 - Pain not controlled
-

TEMPLATE 7: The New Grad “I Don’t Know What to Do” Script

When You’re Overwhelmed

To Charge Nurse/Preceptor: “I need help prioritizing. I have [describe situation]. My gut says [your instinct], but I want to make sure I’m not missing something. Can you help me think through this?”

To Provider: “I’m a new grad nurse caring for [Patient]. I’m seeing [finding] and I’m concerned about [possibility]. I think we need [intervention], but I wanted to discuss with you. What are your thoughts?”

Key: Show your thinking. Don’t just dump the problem. Present your assessment and recommendation, even if you’re unsure.

Communication Quick Reference

Situation	Key Phrase
Need MD to come NOW	“I need you to see this patient now”
Critical lab value	“I have a critical value that needs orders”
Patient declining	“There’s been a change in condition”
Family demanding	“I understand your concern. Let me get the provider”
You’re unsure	“I want to make sure I’m doing the right thing”
Delegating	“I need you to [task]. I’ll check back in [time]”
Refusing unsafe task	“I’m not comfortable with that. Let me get my charge nurse”

Effective communication is 50% of nursing. Master these scripts, and you’ll navigate any situation with confidence.

**Get the complete Clinical Judgment Mastery System at
obiomacare.com**